



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Policy and Procedure

Anticipating and reducing the risk of unpredictable behaviours

Reference #: FSFH-MENH-POL-0027

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health

1. Introduction

The FSFH mental health service is committed to anticipating and reducing the risk of unpredictable behaviours including aggression. It is acknowledged however that at times in mental health settings patients may exhibit risk-prone behaviours, such as verbal and physical aggression, absconding, self-harming, refusing to eat or drink and intimidating behaviours. This document guides staff in anticipating and reducing the risk of violence and aggression to promote a safe and therapeutic environment, reduce the use of restrictive practices and promote a positive patient experience.

2. Terminology

Annexe	A standalone area of the Fiona Stanley Hospital Mental Health Assessment Unit (MHAU) that provides a separate patient care area. The annexe contains the seclusion room, an ensuite, lobby area, courtyard and a sensory room.
Capacity	A patient has capacity if he/she is capable of: - understanding the information being given, the nature of the decision and its consequences - retaining the information as necessary to make the decision

	• using and weighing the information in the decision-making process • communicating their decision in some way. Capacity must always be assessed in the context of the decision that is to be made.
Co-regulation	Provision of external support to assist a patient to develop self-regulation skills and return to baseline through clinician self-awareness and awareness of the patient's emotional needs.
De-escalation	The use of techniques (including verbal and non-verbal communication skills) aimed at defusing anger and averting aggression.
De-escalation room	A therapeutic area used to reduce the patient's distress and propensity to harm self or others; typically, a low stimulus area. It is not the seclusion room or sensory room. The de-escalation room is: • used with the patient's agreement • not locked and the patient may leave the room at any time • the patient can determine how long they stay in the room • only available to one patient at a time
Personal Support Person (PSP)	Person who is supporting someone who is experiencing mental illness. May be any of the following: close family member, carer, nominated person, the parent or guardian of a child and any guardian or enduring guardian adult.
Pro re nata (PRN)	Latin phrase used to indicate when medicines are to be used 'as required'
Sensory interventions	Evidence based therapeutic approaches that utilise the person's sensory inputs of hearing, smell, taste, sight, touch, movement and interoception to promote emotional wellbeing. Interventions utilise sensory equipment, environmental modifications and/or sensorimotor activities to engage the senses (e.g. aromatherapy, movement, sensory kits etc).
Sensory room	A quiet, low stimulus therapeutic space where equipment and activities can be introduced to provide sensory input for calming, alerting and grounding. May also be referred to as a Comfort Room or Quiet Room.

Talk Down intervention	A Safewards intervention that begins with delimiting the situation by moving the patient or other patients to a safe area and maintaining a safe distance; clarifying the reasons for the anger using effective communication; and resolving the problem by finding a mutually agreeable solution.
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3. Policy

- A multidisciplinary team (MDT) approach will be used when anticipating and reducing the risk of violence and aggression.
- Clinicians /AMLO and Peer Workers will endeavour to establish a therapeutic patient relationship as early as possible enabling verbal de-escalation and use of sensory interventions to be used effectively to avoid restrictive practices ¹.
- The MDT team will:
 - utilise a trauma informed, person centred approach
 - support staff emotional regulation and self-management
 - support consumer autonomy through increased self-management
- All mental health staff will attend Aggression Prevention & Intervention (API) Training required for their role. Refer to the SMHS Safety Skills Training Framework.

4. Procedure

4.1. Involving patients in decision-making

- Involve consumers wherever possible in all decisions about their care through the collaborative development of the Treatment Support and Discharge Plan (TSDP) and Risk Assessment and Management Plan (RAMP).
- If a patient is unable or unwilling to participate document reason in the medical record and offer them the opportunity to review and revise the plans as soon as they are able or willing and, if they agree, involve their PSP.
- Ensure the PSP is involved in decision-making for all service users who lack capacity.
- Refer to culturally appropriate Liaison Services including:
 - Lived Experience Coordinator/Peer support worker
 - Aboriginal Mental Health Liaison /Aboriginal Health Liaison
 - Interpreter Services

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4.2. Promoting a safe and therapeutic environment

- Consumers wherever possible will be offered appropriate individual or group based:
 - psychological therapies,
 - physical activities,
 - exploration of emotional regulation and sensory modulation strategies
 - social/leisure pursuits.
- The individual needs of the patient will be recognised including:
 - sensitivity to cultural and spiritual needs
 - language and communication difficulties
 - gender identity.
- All staff will
 - use [Recovery oriented language](#)
 - discuss mutual expectations with patients on admission
 - proactively identify and manage incidents of teasing, bullying, unwanted physical contact, sexual contact or miscommunication between patients.
 - optimise the physical environment (for example, use unlocked doors whenever possible and ensure easy access to outside spaces and privacy).
 - be encouraged to attend education on working with people in withdrawal and craving substances, person centred and trauma informed care.
 - orientate patients to accessible and available resources (i.e sensory items) that can support self-regulation.

4.3. Anticipating patients at increased risk of aggression¹

To anticipate increased risk of aggression and violence all staff will consider:

- reviewing the current TSDP
- the individual patient's mental health diagnosis and/or symptoms which may affect their behaviour (for example, severity of illness, current symptoms and history of violence or aggression)
- any personal factors occurring outside the hospital e.g. family circumstances, financial issues or legal matters) that may affect a patient's behaviour, including Family and Domestic Violence
- substance use history, withdrawal from and craving substances
- trauma history

- intellectual disability, neurodiversity (such as Autism Spectrum Disorder (ASD), Attention Deficit Hyperactivity Disorder (ADHD), cognitive impairment)
- communication difficulties (e.g. hearing deficit, language barriers etc)
- comorbidities such as dual diagnosis, disability.
- the impact of restrictions to a patient's liberty and freedom of movement, such as:
 - being formally detained
 - having leave refused
 - restriction to freedom of communication
 - having a failed detention appeal
 - being in a very restricted environment such as a locked ward

4.4. Assessing and managing the risk of violence and aggression

- When assessing the risk of violence or aggression the MDT will consider previous violent or aggressive episodes as these are associated with an increased risk of future violence and aggression. This may include review of:
 - PSOLIS Alerts and history
 - integrated progress notes
 - previous discharge summaries
 - consider the relevance of one off and older alerts to the current context
- While conducting the assessment the team will:
 - not make negative assumptions based on culture, religion or ethnicity.
 - recognise that unfamiliar cultural practices and customs could be misinterpreted as being aggressive.
 - ensure that the risk assessment is objective and considers the degree to which the perceived risk can be verified.
 - carry out the risk assessment with the patient and, if they agree, their PSP.
- If the assessment identifies the patient is at risk of becoming violent or aggressive, the MDT collaborate with the patient where possible to develop a safety plan (which may be documented on the TSDP) and which considers the following:
 - encouraging patients to recognise their own triggers and early warning signs of violence and aggression and other vulnerabilities, and to discuss and negotiate their wishes should they become agitated.
 - include information regarding the patient's identified triggers and early warning signs.

- symptoms or feelings that may lead to violence and aggression, such as anxiety, agitation, withdrawal and craving substances, disappointment, jealousy and anger, and identification of strategies to manage these symptoms or feelings
- screening for sensory preferences and identifying sensory interventions that moderate arousal and support emotional regulation.
- de-escalation techniques that have worked effectively in the past
- PRN medications
- restrictive interventions that have worked effectively in the past, when they are most likely to be necessary and how potential harm or discomfort can be minimised.
- psychological support and peer support if available to develop greater self-control and techniques for self-soothing.
- encouraging patients to play a key role in the early recognition and reporting of their level of agitation.
- regularly review Risk Assessments and Management Plans (RAMP).
- if transferring to another agency or care setting, or being discharged, share the risk assessment with staff in the relevant agencies or care settings, and with carers.
- provide copy to patient and the PSP

4.5. De-escalation strategies

- De-escalation strategies will take into consideration the patient's cultural and social diversity including:
 - trauma and attachment history
 - family and domestic violence history
 - Aboriginality
 - cultural and linguistic diversity
 - pregnancy/family status
 - disability
 - spirituality
 - sexual diversity and gender identification
 - medical condition
 - age
- Separate agitated patients from others (using quiet areas of the ward, bedrooms, sensory rooms, court yards, or other available spaces) to aid de-escalation, ensuring that staff do not become isolated.

- Use a range of verbal and non-verbal skills and interactional techniques to avoid or manage known 'flashpoint' situations (such as refusing a patient's request, asking them to stop doing something they wish to do or asking that they do something they don't wish to do) without provoking aggression.
- Engage with Aboriginal Mental Health Liaison /Aboriginal Health Liaison to support de-escalation strategies.
- Engage with Lived Experience/peer support workers
- Communicate respectfully and empathetically with the patient at all stages of de-escalation.
- Use sensory based interventions as identified in the patient's safety plan to support self and co-regulation

4.6. De-escalation techniques

- If a patient becomes agitated or angry a staff member will:
 - take the primary role in communicating with them. That staff member should assess the situation for safety, seek clarification with the patient and negotiate to resolve the situation in a non-confrontational manner.
 - use emotional regulation and self-management techniques to control their own verbal and non-verbal expressions of anxiety or frustration (for example, body posture and eye contact) when carrying out de-escalation.
 - use a designated area or room e.g. de-escalation room to reduce emotional arousal or agitation and support the patient to become calm. The seclusion room should not routinely be used for this.
 - use positive communication, distraction, calming, and sensory based techniques and de-escalation strategies at every opportunity and where safe with the view to reducing the need for restrictive interventions.
 - refer to the patient's collaboratively developed TSDP to consider the patient's preferences, if known and if possible, to do so.
 - utilise the Safewards talk down intervention³ when de-escalating as appropriate to the individual situation. Refer to [Appendix 1](#).
 - use the least intrusive level of observation to sensitively monitor changes in mood or composure⁴

4.7. PRN medication

- If safe to do so and aligns with least restrictive practice, non-pharmacological interventions including de-escalation and distraction techniques should be attempted/offered prior to offering PRN medications.
- Refer to the Medication Management of Agitation & Arousal in Mental Health Inpatients policy for the safe and effective use of PRN medications for

pharmacological management of agitation and arousal in mental health inpatients settings.

- This guideline supports the use of six different treatment algorithms based on the age and condition of the patient.

4.8. Strategies for young people

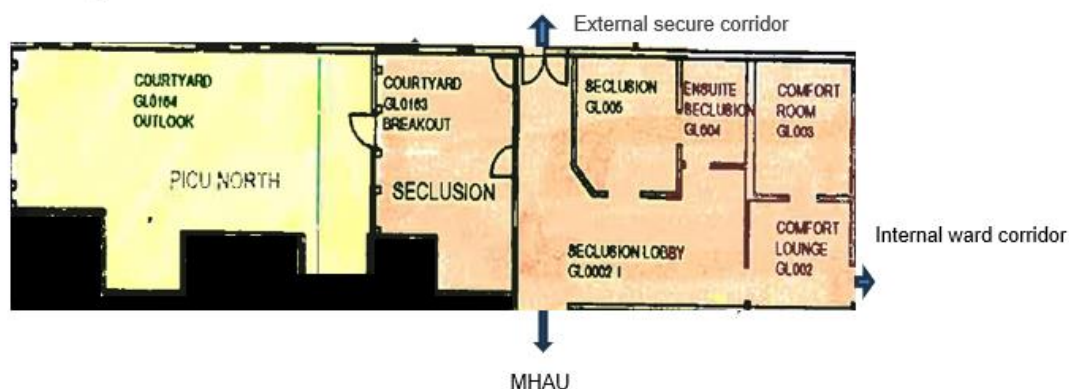
- Additional strategies for consideration on the Youth Unit:
 - co-regulation
 - prompting
 - caring gesture
 - redirection and distraction
 - proximity
- Consider use of sensory rooms/ equipment and sensory modulation techniques or de-escalation room.
- Where behaviour does not settle and there is evidence of escalating risk, medical staff will be requested to attend at the earliest possible time to review the patient's treatment plan and call a Code Black when indicated.
- Reported or observed changes in mental state, and all de-escalation strategies and outcomes must be documented in the patients' medical record and communicated to the treating team.

5. Use of the MHAU annexe for de-escalation

5.1. Annexe area

- The annexe area of the MHAU may be used as a safe environment for the de-escalation of an individual patient.
- Areas of the annexe utilised for de-escalation will include the seclusion room, ensuite, lobby and courtyard. Access to the sensory room and lounge will be restricted.
- When the annexe is in use for de-escalation, the sensory room may be utilised by another patient when it is assessed as clinically appropriate by senior nursing staff.

Diagram 1. Map of annexe



- Access to the annexe area is via:
 - the MHAU through two doors with an airlock system
 - a separate entrance from the internal ward corridor through the sensory room for patients from other wards
 - a separate entrance in the external secure corridor
- Both entrance and exit to the annexe area are restricted to staff with swipe card.

5.2. Indications for use

- Patients who are:
 - dysregulated and unable to communicate their care needs or agree to the use of a de-escalation room
 - disinhibited to maintain their privacy and wellbeing
 - unable to maintain their safety or the safety of others

5.3. Provision of care

- Discuss the rationale for using the annexe area for de-escalation with the patient and seek agreement wherever possible.

Due to the physical layout and restricted access to the annexe, a staff member must always remain with the patient when it is used for de-escalation. When a clinical staff member remains in the annexe with the patient this does not constitute a seclusion event as defined under the Mental Health Act 2014 s. 212.

- Discuss the timeframe for being in the annexe area and inform the patient this will be reviewed with them at regular intervals while in the annexe.
- Voluntary patients must be able to leave the annexe area at will.

- The Patient Observation Form will be taken to the annexe with the patient and the patient's presence in the annexe will be documented including what the patient does whilst in the annexe.
- Patient observations will be completed and documented at a minimum of 15/60 whilst in the annexe.
- Nursing staff will document each time the annexe is used for de-escalation in the patient's medical record including:
 - whether the patient provided agreement
 - time in
 - how the patient responded to de-escalation
 - whether the patient requests to return to the ward but staff determine it is 'too soon'
 - time out.
- Escalate care to the treating team and request medical review where use of the annexe area for de-escalation is not effective for the patient. Refer to local policies:
 - Nursing staff escalation – Mental health
 - Code black mental health
 - Restrictive Practices

6. Staff Support

- The mental health service will ensure systems are in place for identifying, managing and supporting staff who may experience stress resulting from a critical incident. Staff will have access to:
 - Regular clinical supervision / group supervision
 - Post incident support sessions
 - Timely critical incident debriefing
 - Feedback from clinical incident reviews
 - Employee Assistance Program

7. Compliance/Performance Monitoring

- Behaviour incidents will be reported in the Datix Clinical Incident Management System (CIMS). Compliance with this policy will be monitored by Nurse Unit Managers (NUM) in relevant areas via routine monitoring of clinical incident trends.

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- The FSH NUM will maintain a record of use of the annexe area for de-escalation including indications for use and time frames to be reviewed at the Incident Review Meeting.

8. Related Policy Documents

- WA Health:
 - [Safety Planning for Mental Health Consumers MP 0181/24](#)
 - [Safety Planning Procedures for Mental Health Consumers](#)
 - [Principles and best practice for the care of people who may be at risk of exhibiting violent or aggressive behaviour.](#)
- SMHS:
 - [SMHS Safety Skills Training Framework.](#)
 - [Granting and management of leave for mental health inpatients](#)
 - [Employee Assistance Program](#)
 - [Clinical Supervision in mental health](#)
 - [Critical Incident Stress Management](#)
- FSFHG:
 - Clinical risk assessment and management
 - Patient leave and escort – Mental Health
 - Patient Observation – Specialling and close observations
 - Sensory rooms and equipment: safe and therapeutic use
 - Nursing staff escalation – Mental health guideline
 - Interviewing mental health consumers: Managing risk
 - Medication Management of Agitation & Arousal in Mental Health Inpatients
 - Restriction on freedom of communication
 - Visiting in mental health inpatient areas
 - Code Black in the Mental Health
 - Duress alarms and response
 - Vicarious trauma

9. Related Legislation

- [Mental Health Act 2014](#)

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10. Related Standards

- NSQHS
 - Partnering with Consumers
 - Medication Safety
 - Comprehensive Care
 - Communicating for Safety
- Chief Psychiatrists Standards for Clinical Care
 - Care Planning
 - Consumer and Carer involvement in Individual Care

11. References

1. Kuivalainen S, Vehviläinen-Julkunen K, Louheranta O, Putkonen A, Repo-Tiihonen E, Tiihonen J. De-escalation techniques used, and reasons for seclusion and restraint, in a forensic psychiatric hospital. *Int J Ment Health Nurs*. 2017 Oct;26(5):513-524. doi: 10.1111/inm.12389. PMID: 28960738.
2. National Collaborating Centre for Mental Health (UK). Violence and Aggression: Short-Term Management in Mental Health, Health and Community Settings: Updated edition. London: British Psychological Society (UK); 2015. (NICE Guideline, No. 10.) Available from: <https://www.ncbi.nlm.nih.gov/books/NBK356319/>
3. Bowers L, et al. "Reducing Conflict and Containment Rates on Acute Psychiatric Wards: The Safewards Cluster Randomised Controlled Trial." *Int J Nurs Stud* 52 (2015): 1412-1422.
4. Du M, et al. "De-escalation techniques for psychosis-induced aggression or agitation (Review)." *Cochrane Database of Systematic Reviews* 2017, Issue 4. Art No.: CD009922.

12. Authorisation

EXECUTIVE SPONSOR: S5 Service Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	12/2022	CNS, Clinical Improvement	NMPGC	Service 5 Safety Quality and Risk Committee	01/2026
1.1	12/2023	CNS, Clinical Improvement	Minor update no change to review date	Service 5 Safety Quality and Risk Committee	01/2026
1.2	07/2024	S5 SQR Lead	Minor update no change to review date	Service Director – S5	01/2026
2	11/2025	SMHS MH Policy Officer	NMPGC	Service 5 Safety Quality and Risk Committee	11/2027

13. Appendices

13.1. Appendix 1: Safewards Talk Down Intervention

CONTROL YOURSELF

- Act calmly and confidently. Show no fear, subjection or servility
- Have lowered, uncrossed arms and open hands
- Relax face, don't frown or purse lips
- No hesitation or uncertainty or speech
- Breathe deeply and concentrate on situation
- Relax body, no hands on hips or in pockets, don't finger wag or prod
- Have slow and gentle movements
- Don't corner patients, threaten or make false promises
- Don't judge, criticise, show irritation, frustration, anger or be retaliative. This is not personal and it is not about you
- Don't argue or say they are wrong or you are right
- Don't defend or justify yourself
- Show no reaction to abuse or insults directed at you, ignore them or partially agree with them
- Prepare responses in advance to typical insults
- Let patient save face by having the last word so long as they are complying



DELIMIT

- Separate yourself from others/audience/people at risk
- Move to a quiet place, ask to come aside
- Invite patient to sit down
- Establish aid/support/backup
- Maintain distance

CLARIFY

- Ask what's happening, use open questions
- Sort out confusions
- Use patient's name
- Orient patient to time, place and person
- Speak clearly, say who you are, remind of existing relationship, and offer your help
- Wait a second and gain turn
- Paraphrase and check what they have said

RESOLVE

- Request/ask politely, don't demand or be authoritarian
- Give reasons, explain rules, reasoning behind them, be honest, express fallibility (or even agree that it's unfair)
- Give patient opportunity to control him/herself
- Make a personal appeal. remind them of previously agreed strategy
- Deal with the complaint, apologise, make a change
- Outline consequences of different courses of action
- Offer choices and options, leaving power with patient
- Be flexible, negotiate, avoid power struggle, compromise
- Ask if there is anything else you can do or say that will gain their cooperation, ending positively



RESPECT AND EMPATHY

- Show interest, concern and expression congruent with words
- Have a concerned and interested tone of voice
- Listen, hear, acknowledge feelings and needs, be sympathetic
- Take time to hear the patient out, be patient and don't hurry them
- Don't yell over them or shout - wait until they take a breath
- Make eye contact (exercising care not to be confrontational)
- Extend self and thinking to understand patient viewpoint
- Show sincerity, authenticity and genuineness
- Don't tell the patient what they should or should not be feeling
- Don't discount, trivialise or undermine their emotional expression
- No advice giving and no orders, no "If I were you I would....."
- Don't mock patients or treat them as children
- Don't overly smile as this may be seen as condescending
- Answer all requests for information, however they are phrased
- Empathise with feelings, not aggressive behaviour ("I understand you are angry but it is not ok to hit so and so.....")